

Worksheet 1: Common reactions to stressful events

Instructions: Cut these words out and place them in the appropriate space on a timeline or write them on a timeline
(Headings: During and hours after, Days and weeks after, Weeks and months after, Years after).

Shock	Nothing is real
Accepting	Tired
Irritable	Start doing normal everyday activities
Sleep problems	Appetite changes
Anger	Guilt
Fear	Aggressive
Physical aches and pains	Hopelessness
Sense of emptiness	Confused
Feelings of nausea and dizziness	Headaches
Sweating and shaking	Sad
Nervous	Overprotective
Disbelief	Unsure of what to do
Withdrawal from others	Unable to get out of bed
Very busy	Loss of concentration

Complicated grief

Case studies for Activity 1.4

Case 1

Jack owned a textile factory with about 150 employees. The earthquake happened early in the afternoon so most of his employees were inside the building when it collapsed. Jack's wife, Marielle, was in the office. She died on that day, together with about 50 of their employees trapped in the collapsed buildings. Jack was close to the factory and went directly there. He could hear people calling and together with other survivors and bystanders he started to dig their bodies out and help injured people. He kept calling and looking for his wife, but he could not find her. Her body was not retrieved from the rubble until a week later. He did not know if she had died instantly or been alive for some hours or days.

Case 2

George aged 17 had to flee as rebel groups invaded the village and started shooting everyone they saw and burning the houses. George ran to the bush and as he turned to see if his brother and sister were still with him he saw both of them being shot dead on the street. George hid in the bush for many days and had to walk many kilometres to escape the fighting. Two weeks after the event George arrived in a displaced persons camp. He was all alone and had no family or home to which he could return.

Case 3

Henry, a husband and father of two, lost his job two years ago. In the two years since then Henry had become increasingly withdrawn and did not speak much anymore. His wife had tried to encourage him to find a new job, but it was as if Henry had lost his will and energy. The only thing he did was take long walks. One day when Henry's wife was returning from the grocery shop, she saw Henry hanging from a tree. He had committed suicide and had not left any note or words of explanation.

Case studies for topic on PFA

Note on case studies

The following are very generic case studies that could be used in any country and culture. However, please carefully read through them to make sure they are appropriate and realistic to your context. If needed, change the story to ensure your participants can relate to it.

Case study 1

Susi was hit by a car while crossing the street. The driver was turning and did not see her until the last second. He managed to brake but he still could not avoid Susi completely. Susi lost her balance and fell to the ground. You are a Red Cross Red Crescent volunteer trained in psychological first aid and you saw the accident happen as you were walking on the pavement next to the road. Susi did not seem badly hurt, besides a few scratches, and was able to get up. But when she stood up, she felt sick, her heart was pounding and she was trembling violently. She felt dizzy and confused about what had happened and where she was. You, as a volunteer, are at the scene of the accident ready to provide psychological first aid.

Additional suggestions for case stories that can be further developed and adapted

Case study 2

During one of your field trips as a volunteer you are called over to a group of women who are standing outside of a house. You immediately sense that something is wrong, as everyone seems anxious and concerned. They ask you to go into the house to help. When you enter the house, you find a young woman who is screaming and crying and holding her baby who has died in her arms. Her older child is sitting in the corner of the room staring at his mother, and not saying anything.

Case study 3

You are walking to the grocery shop when you see there is a fire a little further down the road in one of the other shops. You rush forward to see how you can help and find that fire fighters have already arrived and have managed to get everyone out of the shop safely. As you look around you see a man crouched on the street rocking back and forth and staring straight ahead. He does not seem to be physically injured, but he is clearly in shock.

Case study 4

You are a volunteer and part of the initial rescue mission during a hurricane. You are walking down what used to be the main street and come across a young woman in distress. She is sobbing uncontrollably, as she stands looking at her house that has been completely destroyed by the storm.

Case study 5

Walking home from a dinner with some friends, you hear a woman screaming. As you turn the corner you see a woman kneeling on the ground, crying and picking up the things from her handbag that are lying all over the road. When you reach her, she tells you that two men attacked her and held a knife to her throat while they stole her money and her phone. The woman is very distressed and keeps saying she thought she was going to die.

Case study 6

One of your best friends comes by in tears and tells you she has just received the news she has breast cancer. She is sobbing uncontrollably and keeps saying she is not ready to die.

Three basic training in PFA

Handout 2: Role-play peer evaluation form

Peer evaluation form		
Action principle and action points	Level of completion (done, not done, partially done)	Comments and examples
LOOK refers to whether the helper looked for		
Information on what had happened and was happening		
Who needed help		
Safety and security risk		
Physical injuries		
Observe/identify immediate basic and practical needs		
Emotional reactions		
LISTEN refers to how the helper		
Approached the person in distress in an appropriate way		
Introduced themselves		
Paid attention and listened actively		
Accepted other person's feelings		
Calmed the person in distress		
Asked about needs and concerns		
helped the person(s) in distress find solutions to their immediate needs and problems		
LINK refers to whether the person in distress was helped to		
Access information		
Tackle practical problems		
Connect with loved ones and social support		
Access services and other help		

Name of participant evaluated: _____

Tool A: PFA action principles Look, Listen and Link

LOOK for

- information on what has happened and is happening
- who needs help
- safety and security risks
- physical injuries
- Observe/identify immediate basic and practical needs
- emotional reactions

LISTEN refers to how the helper

- approaches someone
- introduces oneself
- pays attention and listens actively
- accepts others' feelings
- calms the person in distress
- asks about needs and concerns
- helps the person(s) in distress find solutions to their immediate needs and problems

LINK is helping people

- access information
- tackle practical problems
- connect with loved ones and social support
- access services and other help

Key to filling in the referral form

Section on the referral form Explanation and examples¹

Location	Examples include the name of a specific displaced person (IDP) or refugee camp, or a physical street address. The client/caregiver should be able to physically locate the receiving agency from this information.
Client has been informed of referral (Y/N)	Please explain why the client or caregiver has not been informed of a referral. The consent signature appears towards the end of the form.
Has client been referred to any other organizations (Y/N)	It is helpful for agencies to know about previous referrals to prevent one individual or family being referred several times by multiple agencies for the same service. It also helps guide any future referrals.
Physical health services	Refers to physical health care by doctors, nurses, midwives and community health workers, etc. Please specify in the narrative box whether inpatient or outpatient services are requested.
Mental health services	Category refers to: psychological interventions; clinical management of mental, neurological and substance use (MNS) disorders by specialized and non-specialized health care providers; and support to the caregivers of persons with MNS disorders. Please specify in the narrative box whether inpatient or outpatient services are requested.
Psychosocial activities	This includes community, group and family support activities; child, women and youth friendly spaces; assistance to vulnerable individuals and families; and psycho-education for individuals and families.
Nutrition	Mother-baby groups, promotion of breastfeeding practices, therapeutic-feeding for severe and/or acute malnutrition.

¹ IASC Reference Group for Mental Health and Psychosocial Support in Emergency Settings. (2012). Who is Where, When, and doing What (4Ws) in Mental Health and Psychosocial Support: Manual with Activity Codes (field test-version). Geneva.

Protection support/services	Protection includes mine action and mine risk education, child protection and sexual and gender-based violence (SGBV). Protection support/services covers protection monitoring, specific services for persons with disabilities, survivors of SGBV, survivors of torture, targeted programmes for children associated with armed groups/forces, child labour and case management services for children and SGBV survivors.
Family tracing services	Restoring family links; reunification services; best interest assessment (BIA) and alternative care for unaccompanied and separated children.
Any contact or other restrictions (Y/N)	This question relates to the protection of the client being referred and the principle of "Do No Harm". In some cases, (such as persons with mental health disorders, survivors of sexual and gender based violence, or in cases of child protection), there may be certain restrictions on how to contact the client and how to provide services/support to ensure that you are not causing additional harm. This is important in protection-related cases when the perpetrator may be a family or a community member, and when working with persons with mental health problems to minimize any related stigma and to ensure confidentiality. In such situations, the client may request that she/he be contacted through a close friend, another relative or a trusted community member, or through another medium such as via e-mail, rather than through the telephone. Please write any such concerns or restrictions in the space provided on the form.
Information agencies agree to exchange in follow up	In functioning referral systems, there is often a need for an exchange of information between the referring agency and the receiving agency. In most situations, this is just a confirmation receipt for a referral, but in other situations additional information exchange may be required, whilst respecting the client's wishes for confidentiality (e.g., if one agency is providing case management services and is responsible for coordinating a client's referrals).

1.1.1 Referring agency copy

☐ Routine ☐ Urgent

Date of referral (DD/MM/YY): _____

Referring agency	Receiving agency
Agency/org: _____	Agency/org: _____
Contact: _____	Contact: _____
Phone: _____	Phone: _____
E-mail: _____	E-mail: _____
Location: _____	Location: _____

Client information		
Name: _____	DOB: _____	Nationality: _____
Address: _____		Language: _____
Phone: _____	Sex: _____	ID Number: _____
<i>If client is a minor (under 18 years)</i>		
Name of primary caregiver: _____		Relationship to child: _____
Contact information for caregiver: _____		
Caregiver is informed of referral? ☐ Yes ☐ No (if no, explain) _____		

Background information/reason for referral: (problem description, duration, frequency, etc.) and services already provided	
Has the client been informed of the referral? ☐ Yes ☐ No (If no, please explain below)	Has client been referred to any other organizations? ☐ Yes ☐ No (If no, please explain below)

Services requested:		
☐ Physical health care	☐ Education	☐ Protection services
☐ Mental health services	☐ Shelter	☐ Community centre/Social Services
☐ Psychosocial activities	☐ Material assistance	☐ Family tracking services
☐ Physical rehabilitation	☐ Nutrition	☐ Legal assistance
☐ Financial assistance		

Please explain any requested services:

Consent to release information

(read with client/caregiver and answer any questions before s/he signs below)

I, _____(client name), understand that the purpose of the referral and of disclosing this information to _____(receiving agency) is to ensure the safety and continuity of care among service providers seeking to serve the client.

The service provider, _____(referring agency), has clearly explained the procedure of the referral to me and has listed the exact information that is to be disclosed.

By signing this form, I authorize this exchange of information.

Signature of responsible party: _____

(Client or caregiver if a minor) Date (DD/MM/YY): _____

Details of Referral

Any contact or other restrictions? ☐ Yes ☐ No (If yes, explain) _____

Referral delivered via:

☐ Phone (emergency only) ☐ E-mail ☐ Electronically (e.g., App or database) ☐ In person

Follow-up expected via: ☐ Phone ☐ E-mail ☐ In person By date (DD/MM/YY): _____

Information agencies agree to exchange in follow-up:

Name and signature of recipient: _____

Date received (DD/MM/YY): _____

1.1.2 Client/caregiver copy

☐ Routine ☐ Urgent

Date of referral (DD/MM/YY): _____

Referring agency	Receiving agency
Agency/org: _____	Agency/org: _____
Contact: _____	Contact: _____
Phone: _____	Phone: _____
E-mail: _____	E-mail: _____
Location: _____	Location: _____

Client information		
Name: _____	DOB: _____	Nationality: _____
Address: _____		Language: _____
Phone: _____	Sex: _____	ID Number: _____
<i>If client is a minor (under 18 years)</i>		
Name of primary caregiver: _____		Relationship to child: _____
Contact information for caregiver: _____		
Caregiver is informed of referral? ☐ Yes ☐ No (if no, explain) _____		

Background information/reason for referral: (problem description, duration, frequency, etc.) and services already provided	
Has the client been informed of the referral?	Has client been referred to any other organizations?
☐ Yes ☐ No (If no, please explain below)	☐ Yes ☐ No (If no, please explain below)

Services requested:		
☐ Physical health care	☐ Education	☐ Protection services
☐ Mental health services	☐ Shelter	☐ Community centre/Social Services
☐ Psychosocial activities	☐ Material assistance	☐ Family tracking services
☐ Physical rehabilitation	☐ Nutrition	☐ Legal assistance
☐ Financial assistance		

Please explain any requested services:

Consent to release information

(read with client/caregiver and answer any questions before s/he signs below)

I, _____(client name), understand that the purpose of the referral and of disclosing this information to _____(receiving agency) is to ensure the safety and continuity of care among service providers seeking to serve the client.

The service provider, _____(referring agency), has clearly explained the procedure of the referral to me and has listed the exact information that is to be disclosed.

By signing this form, I authorize this exchange of information.

Signature of responsible party: _____

(Client or caregiver if a minor) Date (DD/MM/YY): _____

Details of Referral

Any contact or other restrictions? ☐ Yes ☐ No (If yes, explain) _____

Referral delivered via:

☐ Phone (emergency only) ☐ E-mail ☐ Electronically (e.g., App or database) ☐ In person

Follow-up expected via: ☐ Phone ☐ E-mail ☐ In person By date (DD/MM/YY): _____

Information agencies agree to exchange in follow-up:

Name and signature of recipient: _____

Date received (DD/MM/YY): _____

1.1.3 Receiving agency copy

☐ Routine ☐ Urgent

Date of referral (DD/MM/YY): _____

Referring agency	Receiving agency
Agency/org: _____	Agency/org: _____
Contact: _____	Contact: _____
Phone: _____	Phone: _____
E-mail: _____	E-mail: _____
Location: _____	Location: _____

Client information		
Name: _____	DOB: _____	Nationality: _____
Address: _____		Language: _____
Phone: _____	Sex: _____	ID Number: _____
<i>If client is a minor (under 18 years)</i>		
Name of primary caregiver: _____		Relationship to child: _____
Contact information for caregiver: _____		
Caregiver is informed of referral? ☐ Yes ☐ No (if no, explain) _____		

Background information/reason for referral: (problem description, duration, frequency, etc.) and services already provided	
Has the client been informed of the referral?	Has client been referred to any other organizations?
☐ Yes ☐ No (If no, please explain below)	☐ Yes ☐ No (If no, please explain below)

Services requested:		
☐ Physical health care	☐ Education	☐ Protection services
☐ Mental health services	☐ Shelter	☐ Community centre/Social Services
☐ Psychosocial activities	☐ Material assistance	☐ Family tracking services
☐ Physical rehabilitation	☐ Nutrition	☐ Legal assistance
☐ Financial assistance		

Please explain any requested services:

Consent to release information

(read with client/caregiver and answer any questions before s/he signs below)

I, _____(client name), understand that the purpose of the referral and of disclosing this information to _____(receiving agency) is to ensure the safety and continuity of care among service providers seeking to serve the client.

The service provider, _____(referring agency), has clearly explained the procedure of the referral to me and has listed the exact information that is to be disclosed.

By signing this form, I authorize this exchange of information.

Signature of responsible party: _____

(Client or caregiver if a minor) Date (DD/MM/YY): _____

Details of Referral

Any contact or other restrictions? ☐ Yes ☐ No (If yes, explain) _____

Referral delivered via:

☐ Phone (emergency only) ☐ E-mail ☐ Electronically (e.g., App or database) ☐ In person

Follow-up expected via: ☐ Phone ☐ E-mail ☐ In person By date (DD/MM/YY): _____

Information agencies agree to exchange in follow-up:

Name and signature of recipient: _____

Date received (DD/MM/YY): _____